

ST. JUDE GENERAL HOSPITAL

Emergency Department / Psychiatric Crisis Unit

Date of Evaluation: August 8, 2026

Patient Name: Michael Torres

DOB / Age: 02/11/1981 | 44 y.o.

Sex: Male

MRN: #102-88-B

Attending Physician: Dr. E. Stavrou, MD

CHIEF COMPLAINT (CC)

"I don't see the point in being here anymore. Everyone would be better off without me." (Brought in by spouse).

HISTORY OF PRESENT ILLNESS (HPI)

The patient is a 44-year-old male brought to the ED by his wife due to worsening depressive symptoms and acute safety concerns. The wife reports the patient has been profoundly depressed for the past 4 months following the loss of his job. He has exhibited severe anhedonia, anergia, a 15-pound weight loss, and severe insomnia. This morning, the wife found a note on his desk expressing goodbye, alongside an accumulation of over-the-counter analgesics. The patient admits to formulating a plan to overdose but states he "didn't have the energy to go through with it yet."

PAST MEDICAL / PSYCHIATRIC HISTORY

Hyperlipidemia. One previous major depressive episode in 2018, treated outpatient with Sertraline (discontinued by patient in 2020). No prior suicide attempts.

MENTAL STATUS EXAMINATION (MSE)

Appearance: Disheveled, poor hygiene, avoiding eye contact. Psychomotor: Significant retardation; movements are slow and heavy. Speech: Latent, low volume, monotonous. Mood: "Empty" and "hopeless." Affect: Blunted and profoundly dysphoric. Thought Process: Linear but impoverished. Thought Content: Expresses feelings of worthlessness and guilt. **Active suicidal ideation with a formulated plan (overdose).** No homicidal ideation. No psychotic features elicited. Insight is fair, judgment is currently impaired by severe depression.

ASSESSMENT

- Major Depressive Disorder, Single Episode, Severe, without psychotic features - F32.2

- Acute Suicidal Ideation with plan

TREATMENT PLAN

1. **Disposition:** Patient meets criteria for involuntary psychiatric hold due to being an imminent danger to himself. Spouse is in agreement with admission. Patient will be transferred to the inpatient psychiatric unit (Ward 4B) for safety stabilization and intensive treatment.
2. **Precautions:** Placed on 1:1 continuous visual observation (Constant Observation/Suicide Watch).
3. **Pharmacotherapy:** Plan to cross-taper to a dual-action antidepressant (e.g., Venlafaxine) pending inpatient psychiatric evaluation. Start Mirtazapine 15mg PO QHS to assist with sleep and appetite.
4. **Clearance:** Medically cleared for psychiatric admission (CBC, CMP, UDS, and ECG are within normal limits).

Dr. E. Stavrou, MD

Electronic Signature Validated